Name	: Ms. NIRMALA GOSWAMI		
Lab No.	: 495383984	Age	: 65 Years
Ref By	: DR KESHAV BHARDWAJ	Gender	: Female
Collected	: 22/10/2025 9:15:00AM	Reported	: 22/10/2025 11:21:16AM
A/c Status	: P	Report Status	: Final
Collected at	: LPL-GURGAON C1/B, Clear View Building Old DLF Colony, Sector-14 Gurgaon, Haryana-122001	Processed at	: LPL-GURGAON C1/B, Clear View Building Old DLF Colony, Sector-14 Gurgaon, Haryana-122001

MC-2133

Test Report

Test Name	Results	Units	Bio. Ref. Interval
COMPLETE BLOOD COUNT (CBC) (DC Detection, Flow Cytometry & SLS)			
Hemoglobin	**9.30**	g/dL	12.00 - 15.00
Packed Cell Volume (PCV)	**30.70**	%	36.00 - 46.00
RBC Count	**3.32**	mill/mm3	3.80 - 4.80
MCV	92.50	fL	83.00 - 101.00
Mentzer Index	27.9		
MCH	28.00	pg	27.00 - 32.00
MCHC	**30.30**	g/dL	31.50 - 34.50
Red Cell Distribution Width (RDW)	**16.20**	%	11.60 - 14.00
Total Leukocyte Count (TLC)	9.06	thou/mm3	4.00 - 10.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils	61.00	%	40.00 - 80.00
Lymphocytes	25.20	%	20.00 - 40.00
Monocytes	7.10	%	2.00 - 10.00
Eosinophils	**6.30**	%	1.00 - 6.00
Basophils	0.40	%	<2.00
Absolute Leucocyte Count			
Neutrophils	5.53	thou/mm3	2.00 - 7.00
Lymphocytes	2.28	thou/mm3	1.00 - 3.00
Monocytes	0.64	thou/mm3	0.20 - 1.00
Eosinophils	**0.57**	thou/mm3	0.02 - 0.50

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MC-2133

Test Report

Test Name	Results	Units	Bio. Ref. Interval
Basophils	0.04	thou/mm3	0.02 - 0.10
Platelet Count	286	thou/mm3	150.00 - 410.00
Mean Platelet Volume	10.3	fL	6.5 - 12.0

Comment

In anaemic conditions Mentzer index is used to differentiate Iron Deficiency Anaemia from Beta- Thalassemia trait. If Mentzer Index value is >13, there is probability of Iron Deficiency Anaemia. A value <13 indicates likelihood of Beta- Thalassemia trait and Hb HPLC is advised to rule out the Thalassemia trait.

Note

1. As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
2. Test conducted on EDTA whole blood

Sparsely distributed RBCs.
anisocytosis ++,
Predominantly Normocytic normochromic RBCs
WBC is normal in number.
There is mild eosinophilia
Platelets are adequate.

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MC-2133

Test Report

Test Name	Results	Units	Bio. Ref. Interval
KIDNEY FUNCTION TEST (KFT)			
Creatinine (Compensated Jaffes reaction, IDMS traceable)	**1.38**	mg/dL	0.55 - 1.02
GFR Estimated (CKD EPI Equation 2021)	**42**	mL/min/1.73m2	>59
GFR Category (KDIGO Guideline 2012)	G3b		
Urea (Urease UV)	**52.00**	mg/dL	17.00 - 49.00
Urea Nitrogen Blood (Urease UV)	**24.28**	mg/dL	8.00 - 23.00
BUN/Creatinine Ratio (Calculated)	18		
Uric Acid (Uricase)	**8.20**	mg/dL	2.60 - 6.00
Total Protein (Biuret)	7.70	g/dL	5.70 - 8.20
Albumin (Bromocresol Green)	4.60	g/dL	3.20 - 4.60
Globulin(Calculated)	3.10	gm/dL	2.0 - 3.5
A : G Ratio (Calculated)	1.48		0.90 - 2.00
Calcium, Total (Arsenazo III)	9.50	mg/dL	8.80 - 10.20
Phosphorus (Molybedate UV)	3.50	mg/dL	2.80 - 4.00
Sodium (Indirect ISE)	140.90	mEq/L	136.00 - 146.00
Potassium (Indirect ISE)	4.47	mEq/L	3.50 - 5.10
Chloride (Indirect ISE)	107.30	mEq/L	101.00 - 109.00

Advise

1. CKD Risk Map (Z1014)
2. Cystatin C, serum (B173)

Note

1. Estimated GFR (eGFR) calculated using the 2021 CKD-EPI creatinine equation and GFR Category reported as per KDIGO guideline 2012.

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MC-2133

Test Report

Test Name	Results	Units	Bio. Ref. Interval

2. eGFR category G1 or G2 does not fulfil the criteria for CKD, in the absence of evidence of kidney damage
3. The BUN-to-creatinine ratio is used to differentiate prerenal and postrenal azotemia from renal azotemia. Because of considerable variability, it should be used only as a rough guide. Normally, the BUN/creatinine ratio is about 10:1
4. Test conducted in Serum

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIPID PROFILE BASIC (Spectrophotometry, Calculated)			
Cholesterol Total	184	mg/dL	<200.00
Triglycerides	132	mg/dL	<150.00
HDL Cholesterol	52	mg/dL	>50.00
LDL Cholesterol,Direct	**125**	mg/dL	<100
VLDL Cholesterol	26	mg/dL	<30.00
Non-HDL Cholesterol	**132**	mg/dL	<130.00

Note

1. Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
2. Lipid Association of India (LAI) recommends screening of all adults above the age of 20 years for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors especially lipid profile. This should be done earlier if there is family history of premature heart disease, dyslipidemia, obesity or other risk factors
3. Triglycerides levels >150 mg/dL in fasting or >175 mg/dL in non-fasting are considered risk modifier for ASCVD risk
4. Test conducted in Serum

Treatment Goals for Lipid lowering therapy*

ASCVD RISK CATEGORY	TREATMENT GOAL	
	LDL-C in mg/dL (Primary target)	NON HDL-C in mg/dL (Co-Primary target)
Low	<100	<130
Moderate	<100	<130
High	<70	<100
Very High	<50	<80
Extreme (A)	<50 (<30 Optional)	<80 (< 60 optional)
Extreme (B)	<30	<60

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MC-2133

Test Report

Test Name	Results	Units	Bio. Ref. Interval

ASCVD Risk Stratification in Indian population*

Indians are at very high risk of developing ASCVD, they usually get the disease at an early age, have a more severe form of the disease and have poorer outcome as compared to the western populations. Many individuals remain asymptomatic before they get heart attack, ASCVD risk helps to identify high risk individuals even when there is no symptom related to heart disease. Risk stratification is important to guide lipid lowering therapy and to identify treatment goals.

**Reference: Lipid Association of India 2023 update on cardiovascular risk assessment and lipid management in Indian patients: Consensus statement IV*

Please Note: *A more formal risk assessment may be used by clinicians according to their personal preferences and familiarity with the risk scores*

Risk factors/markers

Major ASCVD risk factors

1. Age ≥45 years in males and ≥55 years in females
2. Current cigarette smoking or tobacco use*
3. High blood pressure*
4. Low HDL-C

High-risk features

1. Family history of premature ASCVD
2. CKD stage 3B or 4
3. Apolipoprotein B >130 mg/dL
4. Extreme elevation of a single risk factor†
5. Lipoprotein (a) ≥50 mg/dL
6. Metabolic syndrome
7. Non-alcoholic fatty liver disease with fibrosis grade 2 or 3 fibrosis
8. CACS 1-99 and <75th percentile

Risk modifiers

1. Lipoprotein (a) 20-49 mg/dL
2. Impaired fasting glucose (fasting blood glucose 100-125 mg/dL)‡
3. Increased waist circumference (>90 cm in men, >80 cm in women)§
4. hsCRP >2 mg/L¶
5. Plasma triglycerides >150 mg/dL fasting or >175 mg/dL non-fasting
6. Rheumatoid arthritis, psoriasis, and spondyloarthropathies
7. Premature menopause, pre-eclampsia, gestational diabetes, PCOS
8. High polygenic risk score
9. Air pollution
10. Human immunodeficiency virus infection

Risk groups

Low risk	Moderate risk	High risk	Very high risk	Extreme risk
0-1 major ASCVD risk factor, and LDL-C 100-129 mg/dL, and Non-HDL-C 130-159 mg/dL, and Life-time CVD risk <30%#	• 2 major ASCVD risk factors, or • LDL-C 130-159 mg/dL or • Non-HDL-C 160-189 mg/dL or • Low-risk group with ≥1 risk modifier or lifetime ASCVD risk >30%	• ≥3 major ASCVD risk factors, or • LDL-C 160-189 mg/dL or • Non-HDL-C 190-219 mg/dL or • Diabetes with 0-1 major ASCVD risk factors or • 2 major ASCVD risk factor + ≥1 risk modifier or • Any 1 high-risk feature	• Diabetes with target organ damage • Diabetes with ≥2 major ASCVD risk factors • CACS 100-299 or >75th percentile if CACS 1-99 • ≥2 high risk features • Established ASCVD (obstructive or non-obstructive)$ • Heterozygous FH or LDL-C ≥190 mg/dL	**Category A**: • ASCVD with ≥1 feature of high-risk group • CACS ≥300 • Homozygous FH **Category B**: ASCVD with- • ≥1 feature of very high-risk group, or • Recurrent ACS, or • Polyvascular disease, or • Homozygous FH **Recurrent ASCVD event despite LDL-C around 30 mg/dL** These patients require special consideration, please see the text for more details- **Category C**

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MC-2133

Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIVER FUNCTION TEST (LFT)			
AST (SGOT) (IFCC,without P5P)	10.0	U/L	9.00 - 36.00
ALT (SGPT) (IFCC,without P5P)	**9.0**	U/L	10.00 - 49.00
AST:ALT Ratio (Calculated)	**1.11**		<1.00
GGTP (IFCC,L-y-glutamyl-3-carboxy-4-nitroanilide)	18.0	U/L	0 - 38
Alkaline Phosphatase (ALP) (IFCC)	80.00	U/L	30.00 - 120.00
Bilirubin Total (Oxidation)	0.67	mg/dL	0.20 - 1.10
Bilirubin Direct (Oxidation)	0.20	mg/dL	<0.3
Bilirubin Indirect (Calculated)	0.47	mg/dL	<1.10
Total Protein (Biuret)	7.70	g/dL	5.70 - 8.20
Albumin (Bromocresol Green)	4.60	g/dL	3.20 - 4.60
Globulin(Calculated)	3.10	gm/dL	2.0 - 3.5
A : G Ratio (Calculated)	1.48		0.90 - 2.00

Note

1. In an asymptomatic patient, Non alcoholic fatty liver disease (NAFLD) is the most common cause of increased AST, ALT levels. NAFLD is considered as hepatic manifestation of metabolic syndrome.
2. In most type of liver disease, ALT activity is higher than that of AST; exception may be seen in Alcoholic Hepatitis, Hepatic Cirrhosis, and Liver neoplasia. In a patient with Chronic liver disease, AST:ALT ratio>1 is highly suggestive of advanced liver fibrosis.
3. In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or NAFLD, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.
4. In a patient with Chronic Liver disease, AFP and Des-gamma carboxyprothrombin (DCP)/PIVKA II can be used to assess risk for development of Hepatocellular Carcinoma.
5. Test conducted in Serum

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
GLUCOSE FASTING (F) (Hexokinase)	77.00	mg/dL	70 - 100

Note: Test conducted in Plasma

VITAMIN B12 (CYANOCOBALAMIN) (CLIA)	**185.00**	pg/mL	211.00 - 911.00

Notes

1. To differentiate vitamin B12 & folate deficiency, measurement of Methyl malonic acid & Homocysteine levels in serum is suggested
2. The diagnosis of B12 deficiency cannot be solely based on serum B12 levels. Further testing for folic acid, intrinsic factor blocking antibodies, holotranscobalamin (active B12), homocysteine, and/or methylmalonic acid is suggested for symptomatic patients with hematological or neurological abnormalities
3. The concentration of Vitamin B12 obtained with different assay methods cannot be used interchangeably due to differences in assay methods and reagent specificity
4. Test conducted on serum

Comments

Vitamin B12 performs many important functions in the body, but the most significant function is to act as co-enzyme for reducing ribonucleotides to deoxyribonucleotides, a step in the formation of genes. Inadequate dietary intake is not the commonest cause for cobalamine deficiency. The most common cause is malabsorption either due to atrophy of gastric mucosa or diseases of terminal ileum. Cobalamine deficiency leads to Megaloblastic anemia and demyelination of large nerve fibres of spinal cord. Normal body stores are sufficient to last for 3-6 years. Sources of Vitamin B12 are liver, shellfish, fish, meat, eggs, milk, cheese & yogurt.

Decreased Levels

- **Lack of Intrinsic factor:** Total or partial gastrectomy, Atrophic gastritis, Intrinsic factor antibodies
- **Malabsorption**: Regional ileitis, resected bowel, Tropical Sprue, Celiac disease, pancreatic insufficiency, bacterial overgrowth & achlorhydria
- **Loss of ingested vitamin B12:** fish tapeworm
- **Dietary deficiency:** Vegetarians
- **Congenital disorders:** Orotic aciduria & transcobalamine deficiency
- **Increased demand**: Pregnancy specially last trimester

Increased Levels

Chronic renal failure, Congestive heart failure, Acute & Chronic Myeloid Leukemia, Polycythemia vera, Carcinomas with liver metastasis, Liver disease, Drug induced cholestasis & Protein malnutrition

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MC-2133

Test Report

Test Name	Results	Units	Bio. Ref. Interval
URINE EXAMINATION ROUTINE (ROUTINE R/E) (Automated Strip Test, Microscopy)			
Gross Examination			
Colour	Yellow		Pale yellow
Specific Gravity	1.020		1.001 - 1.030
pH	5		5.0 - 8.0
Proteins	Negative		Negative
Glucose	Negative		Negative
Ketones	Negative		Negative
Bilirubin	Negative		Negative
Urobilinogen	Normal		Normal
Blood	Negative		Negative
Leucocyte Esterase	**Present 1+(25 WBC/µL)**		Negative
Nitrite	Negative		Negative
Microscopy			
R.B.C.	0-1 RBC/HPF		0-2 RBC/hpf
Pus Cells	**10-12 WBC/HPF**		0-5 WBC / hpf
Epithelial Cells	2-3 Epi Cells/hpf		0-5 Epi cells/hpf
Casts	None seen		None seen/Lpf
Crystals	None seen		None seen
Others	None seen		None seen

Result Rechecked,
Please Correlate Clinically.

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MC-2133

Test Report

Test Name	Results	Units	Bio. Ref. Interval

DMC-5243

Dr. Nimisha Gupta
MBBS, MD Pathology
Chief of Laboratory
Dr Lal PathLabs Ltd

Dr Rachna Malik
MD, Pathology
Consultant Pathologist
Dr Lal PathLabs Ltd

----------End of report ----------

IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted.•All test results are dependent on the quality of the sample received by the Laboratory. •Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician .•Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.•Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting.•Test results may show interlaboratory variations.•The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s).•Test results are not valid for medico legal purposes.•This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor.•The report does not need physical signature.
(#) Sample drawn from outside source.
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